



TURNKEY KIDNEY CARE PARTNERSHIP

Partner With a National Provider — Own a Slice, Skip the Heavy Lifting

A DaVita / Fresenius joint-venture route: lower capital, turnkey operations, physician equity — for when speed and de-risking matter more than full control.

Prepared for: [Nephrologist / practice name]

THE OPPORTUNITY

The demand is proven. The question is how to capture it.

≈ 4x

higher kidney-failure rate for Black Americans
vs. white Americans

>35%

of dialysis patients are Black (≈13.5% of the
population)

~77%

of Detroit is Black — a large, concentrated
patient base

The bottleneck isn't demand or real estate — it's getting a center stood up and run well. A national-provider JV removes most of that friction.

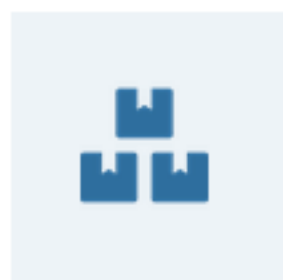
Sources: USRDS 2025; National Kidney Foundation; U.S. Census.

What a national-provider JV brings to the table.



Co-investment

The chain co-funds the build-out, lowering the capital you and your partners put at risk.



Supply leverage

Access to national supply contracts and equipment pricing no independent can match.



Operational platform

Billing, compliance, EHR, and back-office systems refined over decades — running day one.



Scale & brand

An established network, payer relationships, and a name patients and hospitals recognize.

Both DaVita and Fresenius run essentially this same JV playbook — the model below applies to either.

FOR THE PHYSICIAN

Equity upside, without carrying the business.



Financial participation

A revenue stream tied to the center's profitability — not just fee-for-service patient visits.



De-risked entry

The chain absorbs operational and much of the financial risk; you're not betting the practice.



Focus on patients

Operational support frees your time for clinical care while you stay the Medical Director.

This is exactly why the chains favor the JV: with skin in the game, the physician partner is invested in the center's success.

OUR ROLE

We bring the capital and an operator — so you don't.



Investment partner

We fund the physician-side equity contribution and structure the entity, so the capital ask on you is minimal.



A proven operator

An experienced dialysis clinician — community-rooted, MWBE-eligible — to manage scheduling, staffing, and the floor inside the JV.



Community credibility

An operator who reflects the patient base drives the trust and retention that protect both outcomes and the center's economics.

Note: inside a chain JV the operator runs the floor but within the chain's systems — real influence, not full autonomy.

STRUCTURE

How the ownership and money actually work.



Controlling vs. minority

The chain holds the majority (typically 51-75%); physician partners hold the minority. Profits split by ownership percentage.



Steady, de-risked return

Smaller share of a professionally-run, scaled operation — lower variance than going it alone.



Two front doors

DaVita and Fresenius both actively seek physician partners; either can be approached directly by the nephrologist.

Illustrative — chains typically hold 51-75%

What you give up for the safer path.



You gain

- Lower capital at risk
- Turnkey operations & supply pricing
- Faster path to opening
- Lower operational & financial variance
- An established brand and payer relationships



You give up

- Majority ownership & control of the center
- Full autonomy — it's still corporate-managed
- MWBE certification (needs majority ownership)
- A larger share of the upside
- The chance to fix the management model itself

If the goal is independence and fixing how a center is run, this route works against it. If the goal is exposure with less risk, it fits.

NEXT STEPS

If the turnkey route fits, here's the path.

1

Align on goals

Confirm whether speed & de-risking outweigh control for you.

2

Approach the chains

You contact DaVita / Fresenius physician partnerships; we prep the case.

3

Structure & counsel

Negotiate the JV terms with healthcare counsel (Stark / AKS safe harbors).

4

Build & open

The chain stands up operations; the operator runs the floor; ramp begins.

[Your name] • [email] • [phone]

Not legal or financial advice. Physician-ownership dialysis JVs must be structured under Stark Law / Anti-Kickback safe harbors with qualified counsel.